

Diseases of the respiratory tract

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-Diseases of the upper respiratory tract (nose, sinuses, nasopharynx, & larynx)

2. Pharyngitis and *tonsillitis*(*tosilopharyngitis*

Table 1. *Etiology of pharyngitis*

- Fever
- red and/or swollen tonsils
- white or yellow patches on the tonsils
- tender, stiff, and/or swollen neck(swollen lymph nodes)
- painful or difficult swallowing
- Sore throat
- Abdominal pain , vomiting

Tonsillitis

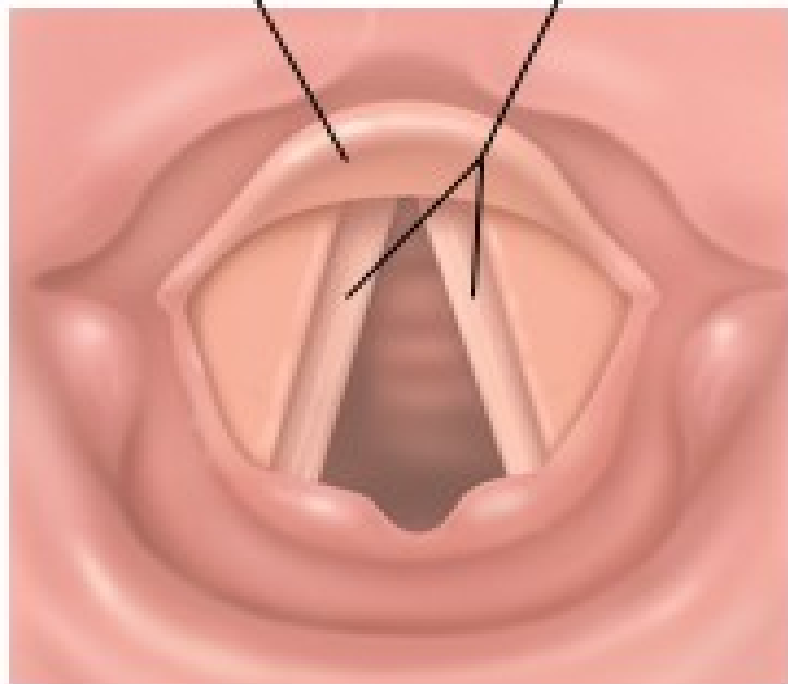


Complications of streptococcal pharyngitis

- Suppurative complications: abscess,
- Nonsuppurative complications: acute rheumatic fever, acute glomerulonephritis.

Epiglottis

Vocal cords



Normal larynx



Inflamed larynx

Vocal cord nodules & polyps

are reactive lesions seen in smokers or individuals who strain their vocal cords (singers)

Singers nodules are bilateral where as polyps are unilateral

Squamous Papilloma and Papillomatosis

Laryngeal squamous papillomas are benign neoplasms, usually located on the true vocal cords, that form soft,raspberry-like proliferations

Carcinoma of larynx

Hyperplasia, dysplasia, carcinoma insitu, & invasive carcinoma

SCC – 95%, rarely adenocarcinoma

Manifested by persistent hoarseness

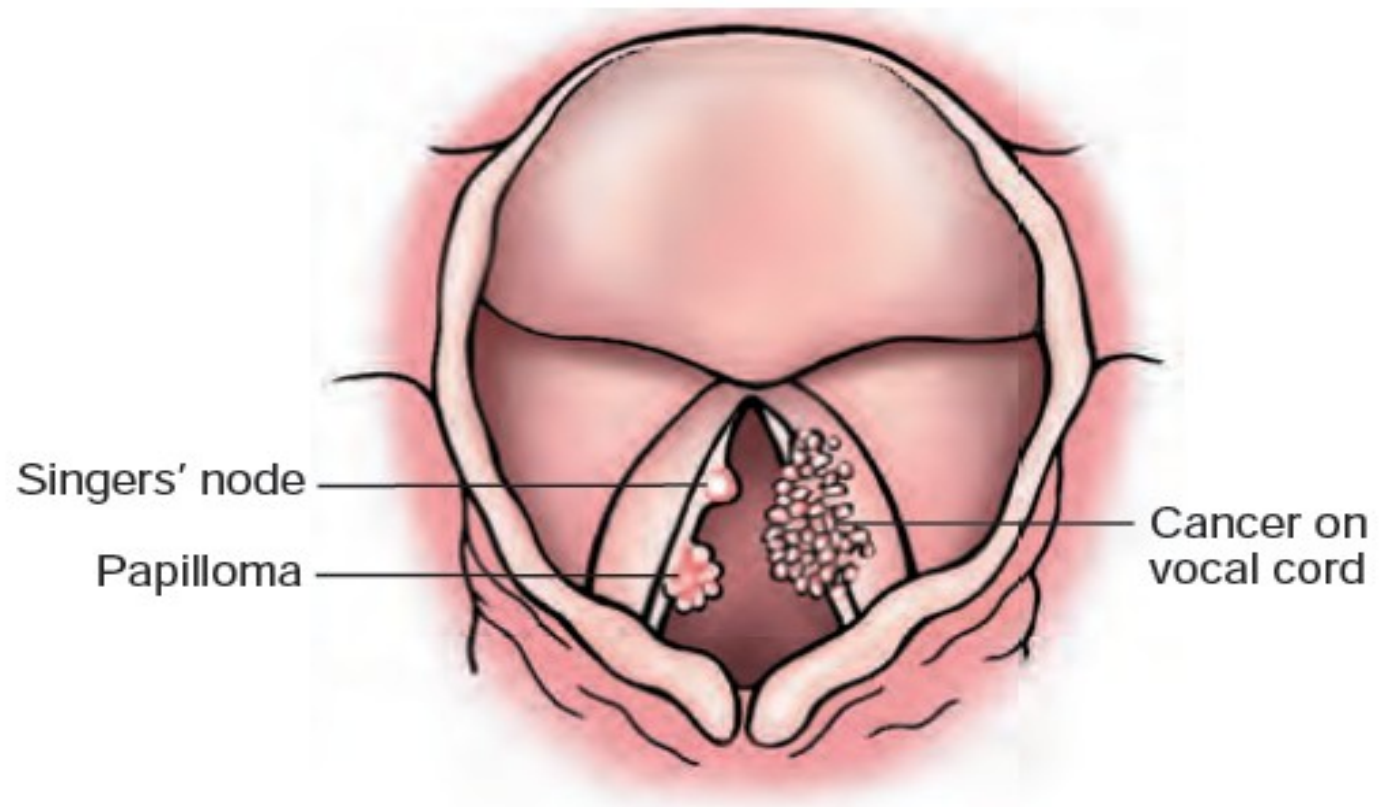


Figure 16-11 Diagrammatic comparison of a singer's nodule, a benign papilloma and an exophytic carcinoma of the larynx to highlight the difference in their clinical appearance.

Diseases of the middle ear

Inflammatory

Otitis Media

- refers to an inflammatory process within the middle ear cleft
- can be either acute or chronic
- disease that persists for more than 3 months should be considered as chronic
- The ET appears to be central to the pathogenesis of all forms of OM

Acute Otitis Media

- AOM is one of the most common infectious diseases seen in children, with its peak incidence in the first 2 years of life
- Most of the population will suffer at least one episode of AOM at some point in their childhood

Contd...

Pathogenesis

- Antecedent viral URTI leads to disruption of eustachian tube function
- Inflammation of the middle ear mucosa results in an effusion, which cannot be cleared via the obstructed eustachian tube
- This effusion provides a favorable medium for proliferation of bacterial pathogens, which reach the middle ear via the eustachian tube, resulting in suppuration
- the majority of patients develop subsequent bacterial colonization, and therefore AOM should be considered a predominantly bacterial infection

Contd...

- *Streptococcus pneumoniae* (up to 40%), *Haemophilus influenzae*, and *Moraxella catarrhalis* (10–20%) are the organisms most commonly responsible for AOM
- Less frequently: group A streptococci, *Staphylococcus aureus*, and gram-negative organisms such as *Pseudomonas aeruginosa*.

Contd...

Symptoms and Signs

- Before the onset of symptoms of AOM, the patient frequently has symptoms of an URTI
- Older children usually complain of earache, whereas infants become irritable and pull at the affected ear.
- A high grade fever .
- Otoscopy classically shows a thickened hyperemic tympanic membrane, which is immobile on pneumatic otoscopy
- otorrhea
- If this occurs, the otalgia and fever often subside.
- At this stage, it is often not possible to visualize the tympanic membrane because of the discharge in the ear canal

acute infection of the middle ear

Complications:

- mastoiditis
- meningitis

Reccurent ear infection may cause chronic secretory otitis media(glue ear), leading to hearing loss

